

Endoscopy — Acute Bleeds & Obstruction

STOOL-TEST

X

STOOL-TEST

X

30% polyps
10% cancer
driving the
urgency

LOW
HEMOGLOBIN
BALLOON

OCULT
BLOOD

OCULT
BLOOD
POSITIVE

GI EMERGENCY FIROUSE

TIPS

BLAKEMORE

BAND LIGATION

GLUE GASTRIC

BAND LIGATION

bleeding esophageal
gastic their manices

ANGRY
ACTIVE
SPURT

ADHERENT
CLOT

ADPIRENT
CLOT

CALM
BASE

CLEAN
BASE

ACTIVE
SPURT
DEMANDING
THERAPY

VISIBLE
VESSEL

THROUGH
SPOT

Epinephrine injection alone
earns small RED X because
it must, must be combined
with a second method.

PPI

OGILVIE
PSEUDO-OBSTRUCTION

RULER
12 CM

NEOSTIGMINE

FIX ELECTROLYTES
& STOP NARCOTICS

NEOSTIGMINE +
DECOMPRESSION
SCOPE READY

X

→ rushing to surgery first.

ALARM
FEATURES

when dyspepsia and dysphagia patients need urgent upper scope:
weight loss, GI bleeding, iron-deficiency anemia, impacted food bolus

ALARM
FEATURES

WHEN DYSPEPSIA AND DYSPHAGIA
PATIENTS NEED URGENT UPPER
• SCOPE: WEIGHT LOSS, GI BLEEDING
• IRON-DEFICIENCY ANEMIA
• IMPACTED FOOD BOLUS

An anemic man or
postmenopausal woman
goes straight to colonoscopy
however tom stoors alarm,
and never just repeats
a stool test.

1. Low-hemoglobin / anemic patient

WHAT YOU SEE

Pale man with a 'LOW HEMOGLOBIN' balloon, left.

WHAT IT ENCODES

Iron-deficiency anemia in a man or postmenopausal woman is GI cancer until proven otherwise — it drives the work-up to colonoscopy (and often EGD). Microcytic anemia + low ferritin = occult GI blood loss source must be found.

2. Occult blood positive paper

WHAT YOU SEE

Sheet reading 'OCCULT BLOOD / OCCULT BLOOD POSITIVE' in the patient's hands, left.

WHAT IT ENCODES

A positive fecal occult blood / FIT result mandates a full colonoscopy, not a repeat stool test. The positive test is the trigger to scope the entire colon to find the lesion.

3. STOOL TEST repeat (red X)

WHAT YOU SEE

Big red Xs over 'STOOL-TEST' signs, lower left.

WHAT IT ENCODES

Repeating a stool test in an anemic or occult-blood-positive patient is wrong — you must go to COLONOSCOPY. The caption notes ~30% have polyps and ~10% have cancer, which is what makes the colonoscopy urgent.

BOARD TRAP

'Repeat the stool test in an anemic patient with positive occult blood' → wrong; an anemic man/postmenopausal woman needs colonoscopy (~30% polyps, ~10% cancer).

4. 30% polyps / 10% cancer urgency

WHAT YOU SEE

'30% polyps 10% cancer driving the urgency' caption, bottom left.

WHAT IT ENCODES

Yield statistics: roughly 30% of these patients have polyps and ~10% have colorectal cancer — the high pretest probability of malignancy is the board's justification for direct colonoscopy over conservative repeat testing.

5. Alarm features banner

WHAT YOU SEE

'ALARM FEATURES — when dyspepsia/dysphagia need urgent upper scope: weight loss, GI bleeding, iron-deficiency anemia, impacted food bolus', top right (and repeated bottom).

WHAT IT ENCODES

Dyspepsia/dysphagia with ALARM features → urgent upper endoscopy: weight loss, GI bleeding/anemia, progressive dysphagia, odynophagia, persistent vomiting, impacted food bolus, age >60 with new symptoms, palpable mass. No 'test-and-treat' for H. pylori until cancer excluded.

6. Band ligation — varices (gold star)

WHAT YOU SEE

Banded esophageal varices, 'BAND LIGATION', gold star on the firetruck, center-left.

WHAT IT ENCODES

GOLD STAR: bleeding esophageal varices → endoscopic band ligation (EVL) is first-line, plus a vasoactive drug (octreotide/terlipressin) and prophylactic antibiotics (ceftriaxone). EVL beats sclerotherapy. Pre-endoscopy: resuscitate, restrictive transfusion to Hb ~7.

7. TIPS / Blakemore / glue backups

WHAT YOU SEE

'TIPS', 'BLAKEMORE', 'GLUE GASTRIC' tools hanging in the bay, center.

WHAT IT ENCODES

Refractory/uncontrolled variceal bleeding → TIPS (transjugular intrahepatic portosystemic shunt) as rescue; balloon tamponade (Sengstaken-Blakemore) as a temporizing bridge. GASTRIC varices are treated with cyanoacrylate glue injection rather than banding.

8. Peptic ulcer stigmata ladder

WHAT YOU SEE

Ladder from 'CLEAN BASE' up through 'FLAT SPOT', 'ADHERENT CLOT', 'VISIBLE VESSEL' to 'ACTIVE SPURT', center.

WHAT IT ENCODES

Forrest rebleeding ladder (high→low risk): active arterial spurt > non-bleeding visible vessel > adherent clot > flat pigmented spot > clean base. High-risk stigmata (spurt, visible vessel, ± clot) get endoscopic therapy; clean base / flat spot do not and can feed early.

9. IV PPI

WHAT YOU SEE

'PPI' block at base of the ladder, center.

WHAT IT ENCODES

High-risk bleeding peptic ulcers → IV PPI (e.g., pantoprazole bolus then infusion) raises gastric pH to stabilize clot. Combined with endoscopic hemostasis. Test and treat H. pylori; stop NSAIDs.

10. Epinephrine alone insufficient (red X)

WHAT YOU SEE

'Epinephrine injection alone earns small RED X — must be combined with a second method', center bottom.

WHAT IT ENCODES

Epinephrine injection achieves temporary tamponade/vasoconstriction but has high rebleed rates ALONE — it must be combined with a second modality: thermal coagulation, hemoclips, or sclerosant. Dual therapy is the standard for high-risk ulcers.

BOARD TRAP

'Treat a spurting ulcer with epinephrine injection alone' → wrong; epinephrine monotherapy rebleeds — combine with clips or thermal/second method.

11. Ogilvie pseudo-obstruction

WHAT YOU SEE

Dilated colon labeled 'OGILVIE PSEUDO-OBSTRUCTION', right.

WHAT IT ENCODES

Ogilvie = acute colonic pseudo-obstruction: massive colonic dilation without mechanical cause, in sick/post-op/elderly patients. Risk of ischemia/perforation rises sharply when cecal diameter >12 cm. First exclude mechanical obstruction (CT) and toxic megacolon.

12. Ruler 12 cm cecum

WHAT YOU SEE

Ruler marked '12 cm' beside the colon, right.

WHAT IT ENCODES

Cecal diameter >12 cm is the threshold of concern for perforation in Ogilvie syndrome and prompts active intervention (neostigmine, decompression) rather than continued watchful waiting.

13. Neostigmine + fix electrolytes

WHAT YOU SEE

Syringe 'NEOSTIGMINE — fix electrolytes & stop narcotics', 'NEOSTIGMINE + DECOMPRESSION SCOPE READY', lower right.

WHAT IT ENCODES

Ogilvie management: correct electrolytes (K, Mg, Ca), stop opioids/anticholinergics, NG/rectal tube. IV neostigmine is the pharmacologic treatment (monitor for bradycardia, have atropine ready). Colonoscopic decompression if neostigmine fails.

14. Surgery first (red X)

WHAT YOU SEE

'rushing to surgery first' (crossed/cautioned) caption, bottom right.

WHAT IT ENCODES

Surgery is NOT first-line for Ogilvie — it's reserved for ischemia, perforation, or failure of medical therapy plus decompression. The board answer is neostigmine + supportive measures first.

BOARD TRAP

'Take the Ogilvie patient to surgery first' → wrong; first-line is neostigmine + electrolyte correction + decompression — surgery only for perforation/ischemia.
